SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Everolimus · neuroendocrine tumours

Progressive unresectable or metastatic pancreatic NET, or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c. · NCCP 00320 · version 6 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Everolimus is a daily targeted tablet used to help control particular advanced NETs. It aims to slow growth; the team balances disease control, side effects and your priorities. This NET guide does not cover the separate kidney-cancer indication.

Schedule
Take the prescribed everolimus dose once daily every day, at the same time and consistently with or without food. There is no planned tablet-free week.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Everolimus

TARGETED THERAPY TABLET

- ☐ Mouth ulcers / stomatitis
- ☐ Infections, including serious opportunistic infections
- ☐ Non-infectious lung inflammation
- ☐ Low blood counts, kidney/liver changes
- ☐ Raised blood glucose and blood fats
- ☐ Rash, diarrhoea, fatigue and reduced appetite
- ☐ Impaired wound healing and radiation-related inflammation

Discuss the exact NET indication, expected control and uncertainty, alternatives, mouth ulcers, infection and lung inflammation, metabolic and kidney/liver effects, interactions, pregnancy precautions and the monitoring plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00320 v6.

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